FAX

Pacific Crest Medical Center
4100 Harbor Mesa Parkway, Orange County, CA
Case Management Department

To	Illumination Health + Home, Recuperative Care Referrals
Fax	888-555-0155
From	Karen Lindqvist, RN, CCM
Phone	714-555-0143
Date	09/01/2026 09:30
Re	Recuperative care referral

MESSAGE

Recuperative care referral. 39-year-old man with right hand cellulitis, improving on oral antibiotics. Unhoused. Independent. History of schizoaffective disorder. Medically cleared.

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Recuperative Care Referral Form

Illumination Health + Home. Submit this form with supporting documents by fax to 888-555-0155.

REFERRING PARTY

Referring Hospital	Pacific Crest Medical Center	**Dept/Floor**	6 East Medical
SW/CM/RN	Karen Lindqvist	**Phone/Pager #**	714-555-0143
Email	klindqvist@pacificcrest.example.org	**Nursing Station #**	714-555-0100
Authorized by	Priya Nandakumar, MD	**Phone #**	714-555-0100

PATIENT INFORMATION

Patient Name	DWAYNE PRUITT	**MRN**	SYN-118903
Date of Birth	07/19/1987	**Ethnicity**	Black/African American
Insurance	Medi-Cal managed care plan	**Preferred Language**	English

Gender: [X] Male [ ] Female [ ] Transgender [ ] Other Last 4 # of Social Security: xxx-xx-0000

Housing Situation: [X] Currently Unhoused [ ] 4+ episodes of homelessness in the past 3 years

HOSPITAL ADMISSION DETAILS

Chief Complaint / Admitting Diagnosis: Cellulitis of right hand

Admit Date: 08/29/2026 Any Wounds? [ ] Yes [X] No

If yes, explain the number, location, size and stage:

Independent with Wound Care? [ ] Yes [ ] No If no, will Home Health be ordered? [ ] Yes [ ] No

Was PPD/TB Test or Chest X-Ray Performed? [ ] Yes [X] No Date: Results:

HISTORY OF HEALTH & LIVING SITUATION

O2	[ ] Yes [X] No	Active Infection	[X] Yes [ ] No
Self Administer All Meds	[X] Yes [ ] No	Ambulatory	[X] Yes [ ] No
Continent of Bowel & Bladder	[X] Yes [ ] No	Assistive Device	[ ] Yes [X] No
Colostomy/Ileostomy	[ ] Yes [X] No	Hospice	[ ] Yes [X] No
Foley Catheter	[ ] Yes [X] No	Car	[ ] Yes [X] No
Diabetes	[ ] Yes [X] No	Service Animal	[ ] Yes [X] No
Insulin	[ ] Yes [ ] No		
Diabetes related Oral Meds	[ ] Yes [ ] No		

Explain (O2 / infection / device / hospice): Right hand cellulitis on oral antibiotics

BEHAVIORAL AND MENTAL HEALTH STATUS

Does the patient have any limitations, behavioral or mental health challenges? [X] Yes [ ] No

[X] Mental Health DX If checked, what type? Schizoaffective disorder, stable on meds

[ ] Non-compliant [ ] Forgetful [ ] Cognitive Impairment [ ] Registered Sex Offender

[ ] Other Explain:

Length of Stay Authorized in Recuperative Care: 30 Days

ILLUMINATION STAFF ONLY

Date Received: _____ Reviewed by: _____

Approved: [ ] Yes [ ] No Date: _____ Authorization #: _____ Reason for Denial: _____

Admission Date: _____ Admission Time: _____ Exit Date: _____

Patient Face Sheet

Printed 09/01/2026 09:30 by Karen Lindqvist

PATIENT INFORMATION

Patient	PRUITT, DWAYNE	**Sex**	M
DOB	07/19/1987	**Age**	39y
Address	NO FIXED ADDRESS	**Language**	English
Phone	NONE	**Marital Status**	Single
Emergency Contact	None listed	**Interpreter**	No

INSURANCE INFORMATION

Primary	Medi-Cal managed care plan	**Member ID**	SYN-M-640257
Secondary	None	**Financial Class**	Medi-Cal Managed Care

ENCOUNTER INFORMATION

Admit Date/Time	08/29/2026 16:40	**Mode of Arrival**	Walk-in
Patient Class	Inpatient	**Unit**	6 East Medical
Admitting Diagnosis	Cellulitis of right hand	**Diagnosis Code**	L03.113, F25.0, Z59.02
Attending	Priya Nandakumar, MD	**Expected Discharge**	09/01/2026

Pacific Crest Medical Center, 4100 Harbor Mesa Parkway, Orange County, CA. Main 714-555-0100.

SYNTHETIC RECORD FOR A HACKATHON - NOT A REAL PATIENT - Claude Impact Lab OC 2026

History and Physical

Priya Nandakumar, MD - Admission date 08/29/2026 - Signed

CHIEF COMPLAINT

Swollen, red right hand for 2 days.

HISTORY OF PRESENT ILLNESS

39-year-old man who cut the back of his right hand on a can lid 4 days ago. Two days of spreading redness and swelling over the back of the hand. No fever. X-ray shows no foreign body or gas. No abscess on ultrasound. Started IV ampicillin-sulbactam.

He has schizoaffective disorder, bipolar type, and says he stopped olanzapine about 2 months ago when he ran out. On admission he is calm and cooperative and denies thoughts of harming himself or others.

PAST MEDICAL HISTORY

- Schizoaffective disorder, bipolar type. Two prior psychiatric hospitalizations, most recent 2024

PAST SURGICAL HISTORY

None.

HOME MEDICATIONS

- Olanzapine 10 mg by mouth nightly (not taking for about 2 months)

ALLERGIES

No known drug allergies.

SOCIAL HISTORY

Unhoused for about 4 years. Sleeps near a rail yard. No current alcohol use. Smokes cigarettes.

PHYSICAL EXAM

General: Awake, alert, in no acute distress.

HEENT: Normocephalic. Mucous membranes moist.

Cardiovascular: Regular rate and rhythm. No murmurs.

Respiratory: Clear to auscultation bilaterally. No increased work of breathing.

Abdomen: Soft, non-tender, non-distended.

Extremities: Back of right hand red, warm and swollen to the wrist. 1 cm healing cut. Full finger motion with mild pain. No fluctuance.

Skin: Warm and dry.

Neuro: Alert and oriented x4. No focal deficits.

Psych: Calm and cooperative. Limited eye contact. Denies suicidal or homicidal thoughts.

ASSESSMENT AND PLAN

- Right hand cellulitis: IV ampicillin-sulbactam, elevate, hand checks.
- Schizoaffective disorder, off medication: monitor. Psychiatry if symptoms emerge.
- Disposition: unhoused. Case management consult.

Case Management / Social Work Note

Karen Lindqvist, RN, CCM - 08/30/2026 15:10

Met with patient. Calm and pleasant. Unhoused about 4 years. Agreeable to recuperative care to finish antibiotics. States he will take his medications. Referral form completed today.

DISCHARGE PLANNING

Item	Status
Housing	Unhoused about 4 years.
Disposition	Recuperative care referral prepared 08/30/2026. Faxed 09/01/2026.
Medications	Reports he can manage his own.
Health plan authorization	Submitted 09/01/2026. Pending.

PRUITT, DWAYNE (MRN: SYN-118903) DOB: 07/19/1987 39y M Pacific Crest Medical Center · 6 East Medical

Authorization Request: Recuperative Care (Community Support)

Submitted 09/01/2026 09:30 by Karen Lindqvist

Health Plan	Medi-Cal managed care plan
Member ID	SYN-M-640257
Service Requested	Recuperative care (medical respite)
Requested Provider	Illumination Health + Home
Days Requested	30
Diagnosis Code(s)	L03.113, F25.0, Z59.02
Status	Pending health plan review
Authorization #	Not yet assigned

CLINICAL JUSTIFICATION

Recuperative care referral. 39-year-old man with right hand cellulitis, improving on oral antibiotics. Unhoused. Independent. History of schizoaffective disorder. Medically cleared.

Laboratory Results

Collected 08/31/2026 05:20

COMPLETE BLOOD COUNT

Test	Result	Reference	Units
WBC	8.2	4.0-11.0	K/uL
Hemoglobin	13.8	12.0-17.0	g/dL
Hematocrit	41.4	36-50	%
Platelets	215	150-400	K/uL

BASIC METABOLIC PANEL

Test	Result	Reference	Units
Sodium	139	135-145	mmol/L
Potassium	4.2	3.5-5.1	mmol/L
Chloride	105	98-107	mmol/L
CO2	24	22-29	mmol/L
BUN	13	7-20	mg/dL
Creatinine	0.9	0.6-1.3	mg/dL
Glucose	92	70-99	mg/dL

TRENDS

Test	Collected	Result	Reference
WBC	08/29, 08/31	12.1, 8.2 K/uL	4.0-11.0

MICROBIOLOGY

Test	Collected	Result
MRSA nares PCR	08/29/2026 18:20	NOT DETECTED
SARS-CoV-2 antigen	09/01/2026 07:45	NEGATIVE

Vital Signs Flowsheet

Date/Time	BP	HR	RR	Temp F	SpO2	O2
08/31/2026 12:00	124/78	88	16	98.4	99%	Room air
08/31/2026 16:00	128/80	92	18	98.6	99%	Room air
08/31/2026 20:00	122/76	86	16	98.2	98%	Room air
09/01/2026 00:00	120/74	84	16	98.0	99%	Room air
09/01/2026 06:00	126/78	82	16	98.3	99%	Room air

Physical Therapy Screen

Brooke Tanaka, PT, DPT - 08/30/2026 11:00 - Signed

Area	Finding
Bed mobility	Independent
Transfers	Independent
Ambulation	300 ft, independent, no device
Balance	Good
Assistive device	None

ASSESSMENT

No mobility deficits.

Discharge recommendation: No PT needs.

Progress Notes

08/30/2026 09:20 Priya Nandakumar, MD Hospital Medicine Progress Note

Hand redness improving. Afebrile. Changed to oral amoxicillin-clavulanate.

Nursing reports the patient was awake most of the night talking to someone who was not there. Psychiatry consult requested.

08/31/2026 15:05 Priya Nandakumar, MD Hospital Medicine Progress Note

Hand continues to improve on oral antibiotics. Medically stable.

Seen by psychiatry this afternoon. See their note. One-to-one observation ordered at 14:20. Olanzapine restarted tonight.

Discharge to a community setting is on hold pending psychiatry.

Nursing Assessment / ADL Flowsheet

R. Vasquez, RN - 08/31/2026 22:40

Item	Assessment
Orientation	Alert and oriented x4. Distracted, responding to voices
Bathing	Independent
Dressing	Independent
Toileting	Independent. Continent of bowel and bladder
Feeding	Independent
Medications	Took first dose of olanzapine with encouragement
Fall risk	Low
Skin	Intact
Isolation	None. One-to-one sitter at bedside for safety
Safety screen	Suicide risk screen POSITIVE 08/31/2026. See psychiatry note. One-to-one observation

NURSING NOTE

Patient pacing and talking to himself. Says, 'He keeps telling me to do it.' Sitter remains at the bedside. Belongings secured.

Psychiatry Consult

Leonard Abiodun, MD - 08/31/2026 14:30 - Signed

Reason for consult: Responding to internal stimuli. Off antipsychotic medication.

Patient reports that for about 3 weeks he has heard a man's voice telling him to step in front of a train and that he deserves to die. The voice has been louder since he stopped olanzapine. He says he has thought about walking onto the tracks near where he sleeps and cannot promise that he would stay safe if he left the hospital. He denies thoughts of harming others.

Mental status: disheveled, guarded, intermittently turning to respond to a voice. Speech soft. Thought process tangential. Mood 'tired.' Affect flat. Insight and judgment poor.

Assessment: schizoaffective disorder, bipolar type, acute exacerbation, with command auditory hallucinations and active suicidal ideation with a plan.

RECOMMENDATIONS

- One-to-one observation.
- Restart olanzapine 10 mg nightly and titrate.
- Involuntary psychiatric hold started 08/31/2026 14:10.
- Transfer to an inpatient psychiatric unit when medically cleared.
- Not appropriate for discharge to a community setting at this time.

Discharge Medication List

Reconciled 09/01/2026

Medication	Dose	Route	Frequency	Notes
Amoxicillin-clavulanate	875/125 mg	By mouth	Twice daily	Through 09/05/2026
Olanzapine	10 mg	By mouth	Nightly	Restarted 08/31/2026

30-day supply to bedside before discharge.

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